

MediLens

*Navigating Global Medicine Intelligence & The Advocacy Toolkit
A Friendly Guide for Patients, Advocates, and Policy Researchers*

1. Welcome to MediLens!

Hello! We are so glad you are here. MediLens was built for you - whether you are a patient looking for life-saving treatments, a community advocate campaigning for health equity, or a policy researcher assembling reports. The 'medicine gap' is a real and urgent global challenge, where essential medications often arrive years late or at excessive costs in many countries. MediLens uses clear, open reference data to make these gaps visible, and gives you a powerful set of AI-driven advocacy tools to do something about it!

2. How the Metrics are Calculated

To make sure our advocacy stands on solid rock, MediLens uses simple, mathematically transparent formulas to calculate lags and price differences. Here is how they work in simple terms:

- How the Approval Timeline Calculates (Lag Days):** Every drug has a 'Global First Approval Date' (usually registered in the USA by the FDA or the EU by the EMA). This is day zero. When a country registers that same drug, we record their 'Local Approval Date'. The Lag Days = (Local Approval Date - Global First Date). For example, if a cancer therapy is approved globally on Jan 1, 2022, and approved in the UK on Jan 1, 2023, the UK lag is exactly 365 days (1 year). A longer lag time highlights slow regulatory reviews or manufacturer delays in filing registrations locally.
- How Global Pricing is Calculated (Ratios):** We convert local medicine pack prices into US Dollars (USD) to establish an honest baseline. We then search our database to find the 'Global Minimum Price' for that exact drug (often found in major competitive generic economies like India or Brazil). The Price Ratio = (Your Local Price in USD / Global Minimum Price in USD). For example, if Metformin costs \$1.20 in your country, but is available in India for \$0.15, the price ratio is 8.0x. Ratios above 10x are flagged in amber, and those above 50x are flagged in red to highlight excessive markups and potential areas for cost negotiation.

3. What You Get When You Click: AI Report Actions

On each Country Dashboard, you will find active buttons powered by the MediLens AI Engine. Clicking these buttons instantly processes the country's local statistics to give you highly customized materials:

-  **Generate Country Policy Brief:** Generates a professional, publication-ready access report. The AI scans the country's access gaps, lag days, and pricing percentile to write a structured case study detailing the current bottleneck, standard regional comparisons, and concrete regulatory policy recommendations.
-  **Assess Shortage Risk:** Evaluates supply chain vulnerability. The AI maps local registration density against global manufacturer pools. It outputs a clinical alert highlighting essential medicines that depend on a single manufacturer, scoring the overall national risk of stockouts and outlining mitigation strategies.
-  **Build Advocacy Action Plan:** Designs a customized, highly practical campaign roadmap. You get a set of target contacts (regulatory offices, ministries), targeted campaign messaging angles based on local gaps, and timelines to structure local patient coalition rallies.
-  **Unregistered Drugs > 'Analyze':** Located next to each drug in the unregistered list. Clicking this generates a deep clinical and economic case study on that specific drug's absence, explaining why it is delayed, what alternative treatments local patients are forced to use, and how much is lost in pricing overcharges.

4. Harnessing the Advocacy Toolkit

The Advocacy Toolkit is your launchpad for real-world impact. When you generate these tools, here is what they provide and how you can use them to drive change in your community:

-  **Policy Brief:** What you get: A formal, professionally structured policy brief outlining access statistics, comparative charts, and economic justifications.
How it helps: Print this out or email it to local health ministers, members of parliament, or medical regulatory boards to present a clear, data-driven argument for policy reforms.
-  **Appeal Letter:** What you get: A fully formatted, persuasive petition letter pre-populated with your country's specific drug pricing and registration gaps.
How it helps: Signed by clinical coalitions, patient families, or local NGOs, these letters are ready to be sent directly to pharmaceutical manufacturers and national drug registrars, demanding pricing concessions or immediate compassionate-use registration.

-  **Supply Chain / Supply Risk:** What you get: A risk assessment matrix identifying essential medicines that rely on single-source active pharmaceutical ingredients (APIs).
How it helps: Present this to hospital procurement boards and health ministries to advocate for diversified bidding and preemptive stock buffers before critical shortages occur.
-  **Campaign & Advocacy Action Plan:** What you get: A practical campaign checklist, complete with social media mobilization templates, key campaign milestones, and press release templates.
How it helps: Provides a clear guide for local patient support groups to coordinate advocacy drives, write news releases, and engage local community networks.

5. Decoding New Drug Radar Badges & Tags

The New Drug Radar uses clear visual badges to help you identify which new treatments represent the most urgent access priorities. Here is what each tag means:

Badge / Tag	What It Tells You	Advocacy Priority Level
WHO EML	Recognized by the WHO on the Essential Medicines List as a critical, basic therapy.	CRITICAL (Must be approved locally!)
 Novel Drug	A newly synthesized chemical entity approved globally within the past 3 years.	HIGH (Monitor local filing progress)
 AI Profile	Our AI has fully compiled active molecules, clinical trials, and global pricing summaries.	INFORMATIONAL (Deep data available)
 Fast spread	Approved in multiple focus markets quickly. Strong manufacturer marketing interest.	MEDIUM (Watch commercial pricing)
 Growing	Registration is spreading steadily across major middle-income regions.	MEDIUM (Benchmark registration lags)
 Slow spread	Approved globally but stalled in only a few markets. High exclusion risk.	HIGH (Immediate target for advocacy)
 No LMIC coverage	Approved only in HICs. High-income exclusive; completely missing in LMIC markets.	CRITICAL (High inequality alert)